The (EU) regulation of health data sharing - ACD

Tjaša Petročnik, TILT, Tilburg Law School
t.petrocnik@tilburguniversity.edu

Lecture recap (1)

Including in healthcare

Data is not an ordinary good	Economic and social value in sharing data for various policy goals
Barriers to data sharing	Incentive alignment problem and insufficient (voluntary) data sharing
Regulatory intervention needed	Balancing of particular and societal interests

Lecture recap (2)



The EHDS: primary use of health data

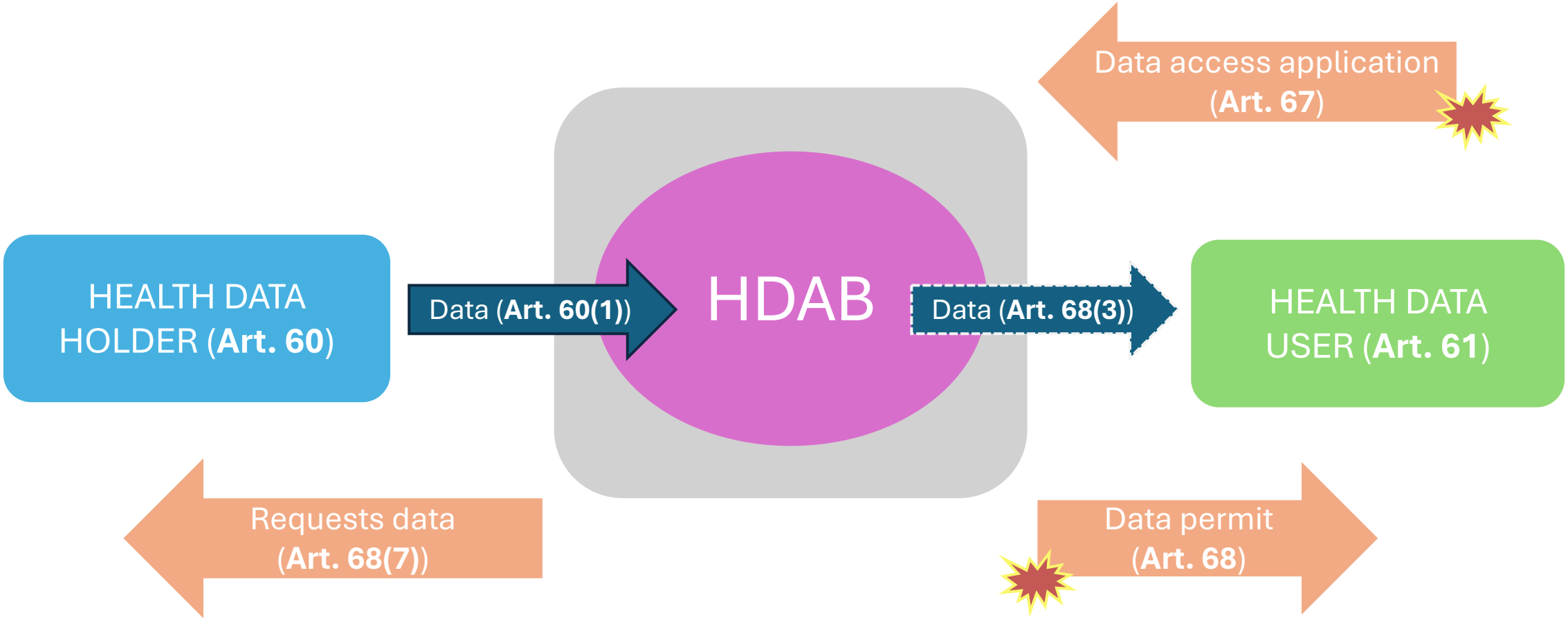
- Priority categories of health data (**Art. 14**)
- Rights of natural persons in relation to primary use of their health data (**Art. 3-10**)
  - Incl. right to insert information into their EHR (**Art. 5**)
- European electronic health record exchange format (**Art. 15**) and interoperability and other requirements for EHRs (**Ch. 3**)
  - Interoperability of wellness apps with EHRs (→ labelling); no automatic sharing (**Art. 47-8**)
- Cross-border infrastructure MyHealth@EU (**Art. 23**)



The EHDS: secondary use of health data

- Reuse of health data for research, innovation, policy-making, education etc. that contributes to the general interest of society (**Rec. 61**)
- Minimum categories of health data for secondary use (**Art. 51**)
  - Incl. data from wellness applications
- Delineation of allowed and prohibited purposes (**Art. 53-4**)
- Administrative procedure and body to manage secondary use (**Art. 55**)
- Infrastructure HealthData@EU (**Art. 75**)



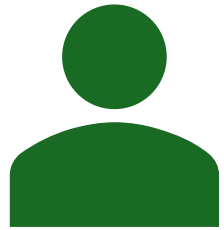

Data access application
(Art. 67)
HEALTH DATA
HOLDER (Art. 60)
Data (Art. 60(1))
HDAB
Data (Art. 68(3))
HEALTH DATA
USER (Art. 61)
Requests data
(Art. 68(7))
Data permit
(Art. 68)

Balancing between public and private interests in secondary use of health data



Public interest:

Harnessing the potential of (existing) data and sharing it widely for societal benefits



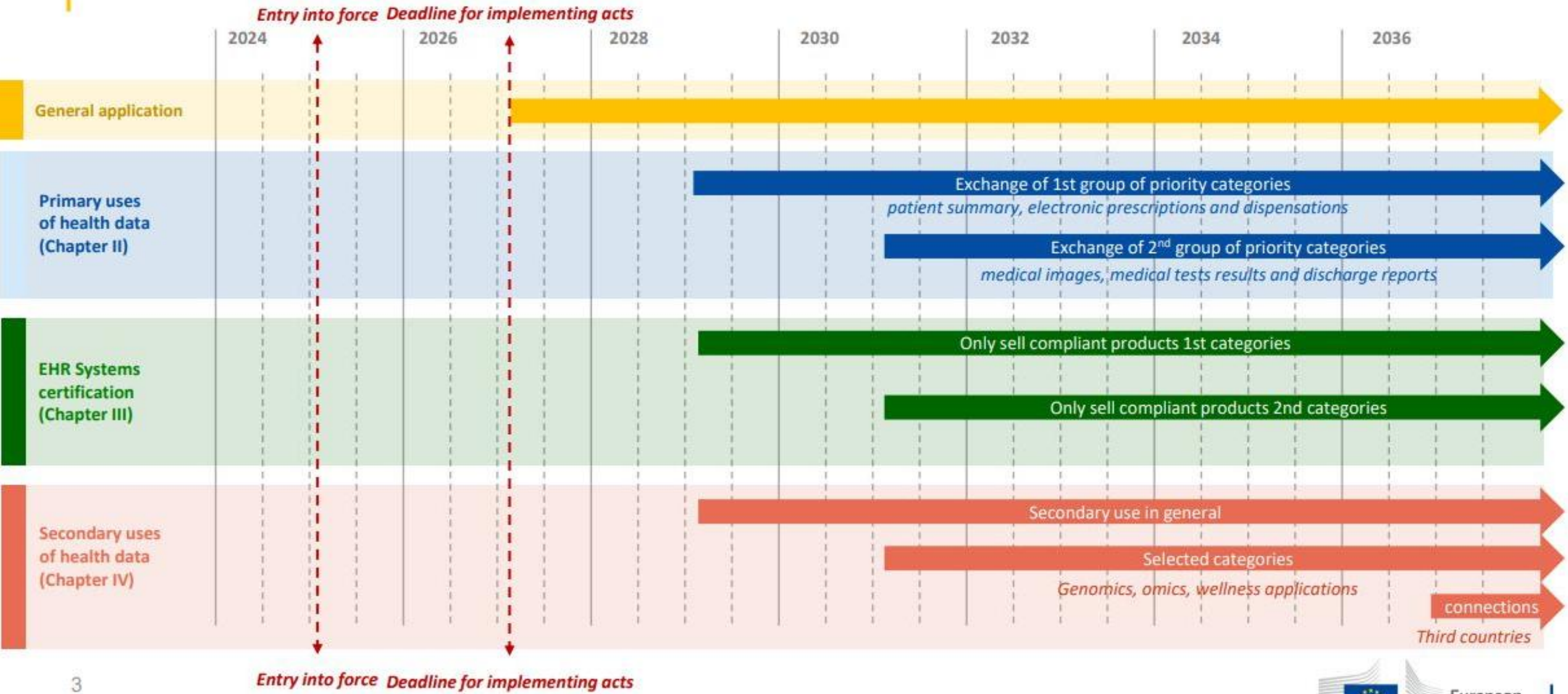
Private interests:

E.g. individual control and autonomy

→ the opt-out clause (**Art. 71**)

Implementation of the EHDS Regulation

Timelines are indicative.



European Commission

ACD: wellness apps and data



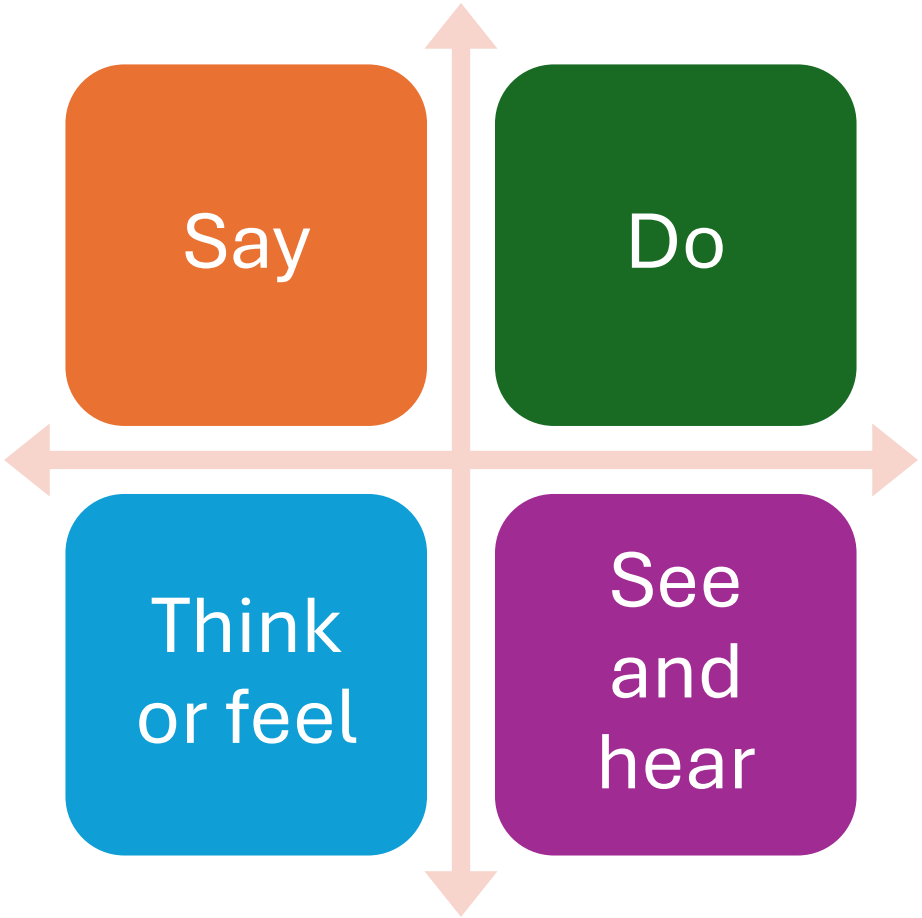


Scenario 1 – empathy mapping

- Wellness app *VitaFit* continuously and passively collects data on users' heart rate, sleep patterns, and physical activity from a smartwatch. It then produces various metrics on app users' wellbeing levels.
- This data not only helps app users manage their (general) fitness; it could also offer important health insights and significantly enhance healthcare provision.
- The Lowlands - a fictional EU Member State - institutes a policy that promotes integration of such wellness app data into Electronic Health Records (EHRs). The idea behind the policy is to give General Practitioners a more comprehensive view of their patients' lifestyle and possible risk factors and enable earlier intervention.



What does the stakeholder ...



Scenario 2 - exam practice

- *Aeolian* is a health insurance company in the Lowlands. It wants to develop an LLM-based coach for preventative lifestyle interventions.
- The coach would provide personalised advice based on individual's health aims and features. It would be available to all residents in the Lowlands and not only to Aeolian's clients.
- Aeolian claims this could importantly reduce healthcare costs and lead to better health outcomes in the country. They say it would also contribute to innovation, as it could lead to discovery of new benchmarks for LLM accuracy.
- To train and test its algorithms and make sure they are safe and effective, Aeolian would like to access users' fitness tracking data from the VitaFit app.

